BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

Check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds	BP, urine protein and wound/ procedure review	Blood pressure before each cycle AND after treatment; urine protein each cycle	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan
Heart checks	Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment
INR only if taking warfarin	Tell the team about warfarin and arrange a baseline INR	INR weekly until the warfarin dose is stable, then before each cycle	Report bleeding or unusual bruising	Only change anticoagulants as instructed	Continue the individually agreed anticoagulation follow-up

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00449 v6a · official regimen

THE WHOLE TREATMENT JOURNEY

FOLFIRI + bevacizumab: treatment and checks

Metastatic colorectal cancer

Day 1: bevacizumab, irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump. A cycle lasts 14 days; your team confirms the next treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00449	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00449	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Blood pressure / urine / wounds NCCP 00449	BP, urine protein and wound/ procedure review	Blood pressure before each cycle AND after treatment; urine protein each cycle	Report bleeding, severe headache or abdominal pain urgently	Additional urine tests and specialist review if abnormal	Confirm wound-healing and procedure plan
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INR only if taking warfarin NCCP 00449	Tell the team about warfarin and arrange a baseline INR	INR weekly until the warfarin dose is stable, then before each cycle	Report bleeding or unusual bruising	Only change anticoagulants as instructed	Continue the individually agreed anticoagulation follow-up

What each check is for

1

14-day treatment cycle

Day ward → home pump → review

FOLFIRI + bevacizumab is used for cancer of the colon or rectum that has spread. Treatment aims to control the cancer; the likely benefit and review plan depend on your individual situation. Surgery may be reconsidered in selected cases after multidisciplinary review.

Ask / act: Confirm the phase, prescription, tests and supportive medicines before starting.

Sources: NCCP 00449

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.33